

I need to write an NDIS high-cost AT report. I am an MS patient and need Bioness funding. If there is a report written by someone else, try looking for it.

Yes. However, the original NDIS High Cost AT Reports actually submitted by others are rarely disclosed due to personal information and copyright issues.

Instead, I looked it up and found some materials worth referring to.

Official NDIS High Cost AT Reporting Standards Actual

MS + Bioness L300 Go Case Study Introduced by Australian Orthotist/AOPA Bioness

Clinical Evidence Data Used by Australian Suppliers

And the important point is that NDIS examiners do not look at the device itself (Bioness), but rather "why it is absolutely necessary for this person."

High Cost AT Report Structure Expected by Actual Examiners

1. Executive Summary (1 page)

- Participant diagnosis

- Current functional limitation

- AT trial outcome

- Recommended AT

- Cost

- Expected outcomes

2. Participant Background MS

- diagnosis

- Disease progression

- Current mobility status

- Environmental factors

- Existing supports

3. Functional Assessment

- Body structure/function impairment

- Foot drop

- Weakness

- Spasticity

- Fatigue

- Balance deficit

- Reduced endurance

- Activity limitation

- Reduced walking distance

- Increased falls/trips

- Reduced community participation

- Participation restriction

- Shopping

- Social activity

- Employment

- Family participation 4.

Current Equipment and Why It Is Insufficient

Equipment Custom Limitation
OTS AFO Poor fit Not effective

Custom AFO Partial benefit Fatigue persists

Walking stick Some stability Does not address foot drop

Physiotherapy Maintenance only Does not provide real time gait assistance

This part is very important.

NDIS always asks.

"Why can't existing supports meet the participant's needs?"

5. Bioness Trial Results (Most Important Section)

In-clinic trial

Observed improvements:

Improved foot clearance

Reduced toe drag

Improved gait symmetry

Improved walking confidence 8-

week community trial

Observed improvements:

Increased daily use

Increased walking participation

Reduced concentration required during gait

Reduced fatigue

Increased confidence

3-week washout

Observed deterioration after device removal:

Increased toe catching

Increased fatigue

Reduced walking confidence

Reduced community participation

This washout data serves as a very strong basis.

6. Alternative Options Considered

The examiner considers this section to be very important.

Alternative Reason not recommended

Continue without AT Unsafe

Walking stick only Does not correct foot drop

Carbon fibre AFO Does not adapt to changing fatigue

Continue physiotherapy only Not sufficient

Bioness L300 Go Most effective

7. NDIS Reasonable and Necessary Criteria

Section 34 justification A.

Related to disability ÿ

MS directly causes:

Weakness

Fatigue

Mobility impairment B.

Effective and beneficial ÿ

Objective improvements demonstrated through:

Clinical trial

Community trial

Washout period

C. Value for money ÿ

Although initial cost is high:

Potential reduction in:

Falls

Carer burden

Future support requirements

Injury risk

Long-term participation outcomes outweigh cost.

D. Most appropriate funding source ÿ

This is specialised disability-specific technology and not the responsibility of another government system.

My Recommended Table of Contents for Submission (Pages 12–18)

Cover Page

Executive Summary

Participant Information

Medical Background

Current Functional Status

Existing Supports

Clinical Assessment Findings

Goal Alignment

Alternative Options Considered

In-Clinic Trial Results

8-week Community Trial Results 3-week

Washout Results

Risk Assessment

Cost Benefit Analysis

NDIS Reasonable and Necessary Criteria

Final Recommendation

Capacity Building Hours

Supporting Documents & Outcome Measures

And actually, I believe the subject I am about to write about is the same MS + Bioness L300 Go patient I worked with previously.

(35-year-old MS, right foot drop, case of submitting data from an 8-week community trial + 3-week washout)

In that case, since you already have substantial data, it is much better to create a new High Cost AT Report for NDIS submission with a high probability of actual approval rather than looking for other people's reports on the internet.

Machine Translated by Google

If you wish, you can write the NDIS High Cost AT Report (Bioness L300 Go, for MS patients) from scratch, which is the 15–20 page level that actual orthotists submit .

This is a letter written by a neurologist. The cost for Bio-Ness is expected to exceed \$40,000, so an exact estimate hasn't been created yet. This is the patient's NDIS goal. Please use this to create a 20-25 page final submission document that includes the modifications you made above.

This question is really important.

In fact, the High Cost AT Report for final submission should be rewritten in "logical blocks" rather than section by section.

After reviewing what I have written so far, grouping the 20 sections into 7 large blocks seems the most natural.

By doing this, the context flows naturally, and redundancy is almost eliminated.

My recommended final rewriting order

BLOCK 1 (approx. 5~6 pages)

Introduction

Write together

Cover Page

Table of Contents

Executive Summary

Why do you use it together?

Already in the Executive Summary

Diagnosis

Goal

Outcome

Recommendation

This is because all of this goes in.

If you write it separately, you will keep having to edit it.

BLOCK 2 (approx. 5~6 pages)

Participant Story

Write together

Participant Information, Living Environment and Disability Impact

NDIS Goals Alignment

Current Supports and Remaining Support Gaps

Why do you use it together?

These three solve one question.

What kind of life is this person living?

Why is the current support insufficient?

Here

Participation

Activity restriction

Low activity level

Connect all of them.

BLOCK 3 (approx. 7~8 pages)

Clinical Justification

Write together

Clinical Assessment Findings

Alternative Solutions Considered

Clinical Reasoning for Device Selection

This is the core of the report.

Here

Assessment

y

Alternative

y

Why FES

y

Why Bioness

y

Why Lower Leg + Thigh

y

Optimisation

until

You must use it without stopping.

If you write this separately

The content keeps repeating.

BLOCK 4 (approx. 7~8 pages)

Community Trial

Write together

In-clinic Trial

Community Trial Methodology

Community Trial Outcomes

Participant / Carer / Neurologist Feedback

Washout Findings

Machine Translated by Google

These five are actually

This is a clinical trial.

There is also only one timeline.

Assessment

y

Clinic

y

Community

y

Outcome

y

Feedback

y

Washout

You have to write it all at once like this

It is much more natural.

BLOCK 5 (approx. 5~6 pages)

Evidence

Write together

Research Evidence

Risk Assessment

Cost Benefit Analysis

These three are also one.

because

Research

y

Risk

y

Value for money

This is because it needs to be connected.

Here

Thesis

Objective outcome and

BLOCK 6 (approx. 4~5 pages)

Funding Justification

Write together

Reasonable and Necessary

Final Recommendation

You must never separate these two.

because

Section 34

y

Orthotist Recommendation

This must continue.

BLOCK 7

Final Document

References

Appendix

I will make this last.

My recommended actual work sequence

Round 1

Block 1

y

examine

Round 2

Block 2

y

examine

Round 3

yyyyy

Block 3

(Most important)

Round 4

yyyyy

Block 4

Round 5

Block 5

Round 6

Block 6

Round 7

Appendix

Round 8

Final Word Formatting

In my opinion, the most important Block

yyyyy

Block 3

Clinical Justification

30%

yyyyy

Block 4

Community Trial

35%

Block 5

15%

Block 6

20%

One more thing I recommend

The document we just created is

It is about a 90-point level.

For the final submission

I want to create a 100-point document.

Therefore, when rewriting each block, I will not simply refine the sentences, but will review the preceding and succeeding sections together to eliminate redundancy and strengthen the logic.

For example, in Block 3, I will redesign Assessment y Alternative Options y Why Bioness y Why Lower Leg + Thigh into a single flow, and in Block 4, I will write Clinic Trial y Community Trial y Outcomes y Feedback y Washout so that they flow together like a single clinical story.

This approach is the most professional and persuasive structure for a High Cost AT Report intended for actual submission.

great.

From now on, I will write **the actual submission version (Final Submission Version) instead of a draft .**

And Block 1 is not simply rewriting the Cover Page and Executive Summary.

This is the part where you make the examiner understand "why this equipment is needed" within the first 2 to 3 minutes.

The principles I will apply while rewriting Block 1 are as follows.

Write based on Progressive MS + Functional Capacity + Participation, rather than "Foot drop".

Minimize the expression "Independent"

Use a professional tone suitable for High Cost AT

Structured so that the conclusion of the entire report naturally emerges in the Executive Summary

BLOCK 1

Cover Page

NDIS High Cost Assistive Technology Assessment Report

Bioness L300 Go Functional Electrical Stimulation System

Lower Leg and Thigh Configuration

Supporting the Preservation of Functional Capacity and Meaningful Participation for a Person Living with Multiple Sclerosis

Participant Details

Item Details

Participant M

Date of Birth To be completed

NDIS Number To be completed

Primary Diagnosis Multiple Sclerosis (MS)

Duration of Diagnosis Approximately 5–6 years

Report Date To be completed

Recommended Assistive Technology

Bioness L300 Go Functional Electrical Stimulation System

Configuration recommended:

Lower Leg Component

Thigh Component

Clinical programming and optimisation

Initial fitting and education

Follow-up review appointments

Ongoing clinical support

Estimated Cost:

To be confirmed (anticipated to exceed AUD \$40,000)

Prepared By

Mee Young Lee

Orthotist

KOrthotics

Provider Number: To be completed

Phone: To be completed

Purpose of this Report

This report has been prepared to support an application for NDIS funding of the Bioness L300 Go Functional Electrical Stimulation (FES) System with Lower Leg and Thigh Components as High Cost Assistive Technology.

The report presents the clinical justification for funding by integrating:

Comprehensive orthotic assessment

In-clinic trial findings

Structured eight-week community trial

Three-week washout findings

Objective outcome measures independently completed by the treating physiotherapist

Participant logbook and feedback

Carer observations

Treating neurologist endorsement

Current peer-reviewed research evidence

The recommendation is based on the demonstrated ability of the Bioness system to preserve functional capacity, reduce the impact of disability-related impairments and enable meaningful participation in everyday life.

Table of Contents

Executive Summary

Participant Information, Living Environment and Disability Impact

NDIS Goals Alignment

Current Supports and Remaining Support Gaps

Clinical Assessment Findings

Alternative Solutions Considered

Clinical Reasoning for Device Selection

In-Clinic Trial Findings

Eight-Week Community Trial

Community Trial Outcomes

Participant, Carer and Neurologist Feedback

Three-Week Washout Findings

Research Evidence

Risk Assessment

Cost Benefit Analysis

Assessment Against the NDIS Reasonable and Necessary Criteria

Final Clinical Recommendation

References

Appendices

Executive Summary

Executive Summary

This report supports funding for the Bioness L300 Go Functional Electrical Stimulation System with Lower Leg and Thigh Components for M, a 35-year-old woman living with Multiple Sclerosis (MS).

M was diagnosed approximately five to six years ago and experiences progressive neurological impairments that significantly affect mobility, endurance, standing tolerance and participation in everyday activities.

Machine Translated by Google

Although she remains able to mobilise within her home and community, her current level of function is maintained through substantial activity restriction, constant compensatory strategies and continuous cognitive effort rather than efficient or sustainable mobility.

To minimise fatigue and maintain safety, M deliberately limits her activity level, avoids prolonged standing whenever possible and carefully plans daily tasks around her fluctuating symptoms. Consequently, the absence of falls during the previous 12 months should not be interpreted as evidence of adequate mobility, but rather as evidence of the significant effort required to avoid situations that increase her risk.

The purpose of the recommended assistive technology is therefore not simply to improve walking, but to reduce the functional consequences of a progressive neurological condition, preserve functional capacity and enable meaningful participation in activities that have gradually been lost due to increasing fatigue and mobility limitations.

To determine the most appropriate intervention, M completed a structured multidisciplinary assessment process consisting of:

- Comprehensive orthotic assessment
- In-clinic Functional Electrical Stimulation trial
- Eight-week community trial using the Bioness Lower Leg and Thigh configuration
- Three orthotic review and optimisation appointments
- Independent outcome measures completed by the treating physiotherapist
- Participant logbook documenting daily use and functional outcomes
- Three-week washout period following device return
- Review by the treating neurologist

This assessment process demonstrated that the Bioness system provided clinically meaningful improvements that extended beyond gait mechanics.

Objective outcome measures showed improvements in functional mobility, walking endurance and dynamic balance. More importantly, these improvements translated into meaningful changes in M's everyday life.

During the community trial, M progressively increased her activity level and re-engaged in household activities that she had previously avoided because of instability and fatigue. Although she initially experienced increased tiredness due to undertaking more activities than usual, after approximately five weeks she reported requiring fewer daytime naps despite maintaining a higher activity level. This suggests improved movement efficiency and reduced energy expenditure rather than reduced activity.

A particularly meaningful outcome was M's return to cooking and baking. Prior to the trial, she avoided these activities because she was unable to tolerate prolonged standing and repetitive lower limb movements. Following optimisation of the Lower Leg and Thigh configuration, she was able to stand for longer periods, move more efficiently within the kitchen and comfortably perform repeated knee flexion and extension during meal preparation. These changes represent restoration of meaningful participation within her home rather than simply improved gait.

The participant also reported that episodes of knee hyperextension were significantly reduced while using the system and that associated knee pain was absent throughout the active trial period. During the subsequent three-week washout period, intermittent knee hyperextension and knee pain returned, together with increased effort during walking and greater reliance on activity pacing. These findings strengthen the conclusion that the functional improvements observed during the trial were directly attributable to the Bioness system.

The recommendation is further supported by a formal letter from M's treating neurologist, who documented meaningful improvements in mobility, walking endurance, confidence and quality of life while using the system, and endorsed continued access to maximise functional capacity and community participation.

Current peer-reviewed evidence supports the use of Functional Electrical Stimulation for appropriately selected people living with Multiple Sclerosis. However, this recommendation is not based on diagnosis alone. It is based on

the convergence of published evidence with M's individual clinical outcomes, multidisciplinary assessment, objective physiotherapy measures, participant logbook, neurologist endorsement and washout findings.

In my clinical opinion, the Bioness L300 Go Functional Electrical Stimulation System with Lower Leg and Thigh Components is the most appropriate assistive technology solution to address M's disability-related functional limitations.

The recommendation is based not on the purchase of advanced technology, but on the demonstrated ability of the system to preserve functional capacity, increase meaningful participation, reduce the physical and cognitive burden of mobility and delay increasing reliance on informal and funded supports in the context of a progressive neurological condition.

Rewriting Evaluation (Block 1)

This version has been improved in the following ways compared to the previous version.

Shifting the core message from "walking improvement" to "maintaining function and participation," and unifying terminology around "functional capacity" and "meaningful participation" instead of "independence."
Accurately reflects M's actual state (low activity level, activity avoidance, continuous reward strategy)
Structured so that the logic and conclusion of the entire report can be understood by reading only the Executive Summary.

If you rewrite Blocks 2 through 6 with the same tone and logic based on this level, the final document will be a consistent High Cost AT Report for actual submission.

Branched from Branch · NDIS High Cost AT Report

Tue, Jun 30 at 11:38 PM Please
make corrections to this Block 1 if necessary for other parts of muscle strength.

Yes. Based on the information gathered so far, the Muscle Strength (MMT) section must be revised in the final version.

Because in the current report

Ankle dorsiflexion 0/5

Only the emphasis is on,

In reality

Physiotherapist: 0/5

Neurologist: 3/5 (Using Bioness)

Neurologist Letter: DF strength improved from 3/5 ÷ 4-/5

Results from different points in time exist like this.

However, this is not a problem; rather, it can serve as strong evidence to explain the Bioness effect.

Current version issues

Currently, in the Clinical Assessment

Ankle dorsiflexion 0/5

Ankle eversion 0/5

Ankle inversion 1/5

Plantarflexion 3/5

Only [something] is written.

If the NDIS examiner looks at the Neurologist Letter together

You might have a question like this.

Recommended editing method

It is not about changing MMT

Explain the distinction between the Assessment timeline and the Neurologist Review timeline.

You must do it.

Recommended Revision of Block 1 Executive Summary

Current sentence

Clinical assessment identified severe dorsiflexion weakness.

y

suggestion

Clinical assessment identified severe right ankle dorsiflexion weakness, with physiotherapy assessment documenting dorsiflexion strength of 0/5 at baseline. Subsequent neurologist review during active use of the Bioness system documented improvement in dorsiflexion activation, supporting the functional gains observed throughout the community trial.

Modify Block 3 (Clinical Assessment Findings) later

today

Ankle dorsiflexion 0/5

y

suggestion

Muscle Strength Assessment

Baseline Physiotherapy Assessment

Muscle Group Strength

Ankle Dorsiflexion 0/5

Plantarflexion 3/5

Eversion 0/5

Inversion 1/5

Clinical assessment demonstrated profound weakness affecting ankle dorsiflexion and eversion, contributing to foot drop, reduced foot clearance and compensatory gait strategies.

Neurologist Findings During Active Use of the Bioness System

The treating neurologist subsequently documented improved dorsiflexion activation during active use of the Bioness system.

Neurological review recorded:

Right ankle dorsiflexion improved from 3/5 to 4-/5.

Whilst differences in grading may reflect variation between assessors and testing conditions, the findings are consistent with the functional improvements observed throughout the community trial.

Should I add it to the Executive Summary?

recommend not putting numbers (0/5, 3/5, 4-/5) in Block 1.

Executive Summary

Outcome

Participation

Recommendation

This is a place that only shows...

The MMT figures are too detailed.

My recommendation

Block 1

No modifications

(Maintain current state)

Block 3 Clinical Assessment Findings

yyyyy Must be corrected

Add structure below

Baseline Physiotherapy Assessment

y

MMT 0/5

y

Community Trial

y

Neurologist Review

y

DF activation improved 3/5

y 4-/5

y

Consistent with observed functional gains, in my opinion

The strongest argument in the current report is

It is not that "DF has improved."

rather

Low activity level

y

Activity avoidance

y

y

Activity increase

y

Reduced naps

y

Standing tolerance

y

Baking

y

Washout deterioration

This is the logic.

Therefore, it is much stronger to use MMT only as supplementary evidence and continue to place the central axis of the report on Functional Capacity + Participation.

In other words, the best strategy is to keep Block 1 as it is and incorporate MMT-related modifications when rewriting Block 3 (Clinical Assessment Findings).